

Policy on the Content of the Discharge Summary

«Hospital Name»

Document control

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Prepared by	Medical Superintendent Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

Statement of intent

«Hospital Name» defines the content of the discharge summary: provided at the time of discharge, signed and acknowledged, standardised in content, written in an understandable manner with follow-up and medication instructions, incorporating urgent-care instructions, and including cause of death where applicable.

1. Purpose

This policy explains what «Hospital Name»'s discharge summary contains and how instructions are written: provision and acknowledgement at discharge, standardised minimum content, understandable follow-up and medication instructions, urgent-care instructions, and cause of death in case of death.

The discharge process itself — planning, coordination, LAMA guidance, and that a summary is given — is covered in the hospital's other policies.

2. Scope

This policy applies to treating doctors and doctor members of treating teams who prepare and sign discharge summaries, and to all patients and families receiving a discharge or death summary at «Hospital Name».

3. Policy standards

A discharge summary is provided to the patient at the time of discharge, signed by the treating doctor or a doctor member of the treating team, and acknowledged by the patient or family. The discharge summary has standardised minimum content.

The summary contains follow-up advice, medication and other instructions in an understandable manner, incorporates instructions about when and how to obtain urgent care, and, in case of death, includes the cause of death.

4. Non-negotiable rules

- Do not issue a discharge summary without the signature of the treating doctor or a doctor member of the treating team, or without a recorded acknowledgement of receipt by the patient or family.
- Do not omit any of the standardised minimum content fields from a discharge summary without recording the reason it is not applicable.
- Do not write medication frequencies using abbreviations such as BD, TID or QID — write them in plain words the patient can understand.
- Do not omit urgent-care instructions specific to the patient's diagnosis and condition, or the hospital or doctor contact number, from the discharge summary.
- Do not issue a death summary without stating the cause of death, or without documenting a post-mortem when the cause is unclear.

5. What we do

5.1 Provide and acknowledge the discharge summary

A discharge summary is provided to the patient at the time of discharge. The discharge summary is signed by the treating doctor or a doctor member of the treating team. The patient or family acknowledges receipt of the summary, and this acknowledgement is recorded with date and name of the person receiving it.

5.2 Standardise discharge summary content

Discharge summaries have standardised minimum content: patient's name; unique identification number; name of the treating doctor; date of admission and date of discharge; reasons for admission; significant findings, diagnosis and the patient's condition at discharge; investigation results; any procedure performed; medication administered; and any other treatment given. The summary may also name other consultants involved in treatment.

5.3 Write understandable follow-up and medication instructions

The discharge summary contains follow-up advice, medication and other instructions in an understandable manner. Instructions are explained to the patient and/or relatives in a language and manner they understand. Medical abbreviations such as BD, TID and QID are

not used; frequencies are written in plain words — for example morning and night, three times a day. Instructions include safe and effective use of medical equipment at home where applicable, and wound-care or pressure-ulcer care instructions for post-operative patients where relevant.

5.4 Incorporate urgent-care instructions

The discharge summary incorporates instructions about when and how to obtain urgent care, specific to the patient's diagnosis and clinical condition at discharge — for example development of fever, or bleeding or discharge from the operative site. Instructions may state what medicines to take, when to consult a doctor, how to seek medical help, and the contact number of the hospital or doctor. These instructions are explained to the patient and/or relatives in a language and manner they understand.

5.5 Include cause of death where applicable

In case of death, the summary of the case includes the cause of death. When the cause of death is not clear, a post-mortem is performed — for example in a medico-legal case — and documented.

6. Stop-work authority

Not applicable. This standard has no stop-work authority. Gaps in discharge summary content, signature, acknowledgement, or urgent-care instructions are addressed through the hospital's quality monitoring and CAPA process (Section 8), not through an immediate stop-work trigger.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for the standardised discharge summary template and content requirements.
Treating Doctor	Prepares, signs, and explains the discharge summary, including urgent-care instructions, in an understandable manner.
Nursing Superintendent	Supports acknowledgement recording and equipment/wound-care instruction delivery.
Quality Coordinator	Audits this policy; holds discharge summary templates and completed-summary samples.

8. Quality monitoring

Monitoring area	What is monitored
Provision and acknowledgement	Signed discharge summary issued at

	discharge; acknowledgement of receipt with date and name.
Standardised content	Current standardised discharge-summary template; completed summaries with all minimum content fields.
Follow-up and medication instructions	Summaries with plain-language medication frequencies; equipment and wound-care instructions where applicable.
Urgent-care instructions	Urgent-care section specific to diagnosis/condition at discharge, with hospital or doctor contact.
Cause of death	Death summary stating cause of death; post-mortem documentation when required.

9. Training and staff acknowledgement

All treating doctors and doctor members of treating teams shall be trained on the standardised discharge summary content, plain-language instruction requirements, and urgent-care instruction content in this policy, at induction and periodically.

I have read the Policy on the Content of the Discharge Summary of «Hospital Name». I will follow the processes described.

10. Distribution

This policy shall be available to all treating doctors, department heads, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
MLC	Medico-Legal Case
NABH	National Accreditation Board for Hospitals and Healthcare Providers

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means the objective element is starred and requires documented evidence.

Objective Element	Level	Traceability to this policy
AAC.13.a	Commitment	Sections 3 and 5.1 address provision and acknowledgement of the discharge summary.

AAC.13.b	Commitment	Sections 3 and 5.2 address standardised minimum content.
AAC.13.c	Commitment	Sections 3 and 5.3 address understandable follow-up and medication instructions.
AAC.13.d	Commitment	Sections 3 and 5.4 address urgent-care instructions.
AAC.13.e	Commitment	Sections 3 and 5.5 address cause of death in case of death.

13. Required records/evidence checklist

Provision and acknowledgement — AAC.13.a (Commitment)

- Signed discharge summary issued at discharge.
- Acknowledgement of receipt by patient or family (with date and name).

Standardised content — AAC.13.b (Commitment)

- Current standardised discharge-summary template.
- Completed summaries with all minimum content fields.

Follow-up and medication instructions — AAC.13.c (Commitment)

- Summaries with plain-language medication frequencies.
- Equipment and wound-care instructions where applicable.

Urgent-care instructions — AAC.13.d (Commitment)

- Urgent-care section specific to diagnosis/condition at discharge, with hospital or doctor contact.
- Record of explanation to patient/relatives.

Cause of death — AAC.13.e (Commitment)

- Death summary stating cause of death.
- Post-mortem documentation when cause is unclear or MLC.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. AAC.13.

Disclaimer

This policy reorganises the supplied AAC.13 objective-element wording and Guidebook interpretation into plain-language guidance for «Hospital Name». It does not replace the NABH Accreditation Standards for Hospitals, 6th Edition, or the Guidebook, which remain the authoritative source in case of any conflict.

This policy is intended for accreditation and internal governance purposes and is not a statement of, or substitute for, compliance with any specific statute; «Hospital Name» remains separately responsible for meeting all applicable statutory and regulatory requirements, including medico-legal reporting obligations.

This policy shall be reviewed at least once every year, or earlier if NABH standards change.

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